SACTCHECK · TREATMENT PASSPORT

Lutetium-177 oxodotreotide (Lutathera / PRRT)

A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00642/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

My diagnosis, plan and priorities

Complete these pages with your team. Leave an item blank if it does not apply or is not yet known. This personal record does not replace your prescription, clinical notes or consent discussion.

My NET diagnosis

Primary site

Pathology wording / differentiation

Grade

Ki-67 result and sample date (if reported)

Stage / sites involved

Functioning or nonfunctioning / hormone syndrome

SSTR scan result and date

Diagnosis explained by / date

My goals and treatment sequence

What matters most to me

Treatment aim

Current medicines and dose / written prescription location

Other planned surgery, radiation, liver-directed or hormone treatment

Next decision or scan review

Lead team and coordinating nurse

Education informed by NCCN Guidelines for Patients: Neuroendocrine Tumors (2026); written for this Irish treatment context. Your team confirms your individual plan.

My tests, symptoms and support

Safety blood tests, scans to assess treatment, and tests for hormone symptoms have different purposes. Record only the tests agreed for you.

Test and purpose	Due / booking contact	Result explained by / date	Agreed next step

Do not convert every NET marker into a routine test. Record the individual plan.

Symptoms and support

Usual bowel pattern / flushing / glucose symptoms

Changes and impact on daily life

Hormone symptom rescue plan if prescribed

Nutrition / weight / enzyme support

Other medicines and supplements

Questions for my team

Education informed by NCCN Guidelines for Patients: Neuroendocrine Tumors (2026); written for this Irish treatment context. Your team confirms your individual plan.

My PRRT appointments and safety plan

Complete these pages with your team. Leave an item blank if it does not apply or is not yet known. This personal record does not replace your prescription, clinical notes or consent discussion.

PRRT only

Last long-acting SSA / next planned dose

Short-acting SSA stop / restart plan

Blood-test dates between administrations

Radiation-safety restrictions and exact end dates

Travel / household / pregnancy or child-contact plan

Nuclear-medicine urgent contact

Four planned administrations; dates, activity and suitability are confirmed individually.

Keep the nuclear medicine team's written instructions and restriction dates with this record. Do not use a general leaflet to calculate when your personal restrictions end.

Education informed by NCCN Guidelines for Patients: Neuroendocrine Tumors (2026); written for this Irish treatment context. Your team confirms your individual plan.

NCCP 00642 V2 · ONE CYCLE, ONE PAGE

Lutetium-177 oxodotreotide (Lutathera / PRRT)

Cycle number: _____ Start date: _____ Next review: _____

Record administrations T1–T4, blood tests, scan appointments, SSA timing and exact radiation-precaution end dates. A stamp records an administration; it does not approve the next one.

Lutetium-177 oxodotreotide · Receptor-targeted radiopharmaceutical

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	☐ None	☐ Queasy but eating and drinking	☐ Intake reduced: contact the team	☐ Unable to maintain intake: urgent review	☐ Collapse / severe dehydration: 112/999
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999
Diarrhoea	☐ Usual pattern	☐ Up to 3 extra stools above normal	☐ 4–6 extra stools/day or at night: call	☐ 7+ extra stools/day or dehydration: urgent review	☐ Collapse / severe dehydration: 112/999
Bleeding / bruising	☐ None	☐ New small bruise / brief bleed: report	☐ Repeated bleeding: call today	☐ Bleeding that will not stop: urgent help	☐ Heavy bleeding / collapse: 112/999

Lysine / arginine amino acids · Supportive infusion

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
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Dates / doses missed or changed / symptoms affecting daily life / questions:

> Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

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